



Comprehensive Risk Screening, Assessment, Care Planning and Intervention Policy

Scope

Site	Department, Division or Operational Area	Applicable to
Royal Perth Bentley Group (RPBG)	Inpatient Units, Day procedural areas and satellite units	Medical, Nursing and Midwifery, Allied Health, Peer Workers (MH) and Support Staff

Policy statement

The purpose of this policy is to define the RPBG approach for screening, assessment, care planning, documenting and delivering Comprehensive Care (CC) to all patients.

Note for this document:

- Patients in general health settings, women in maternity services and consumers in mental health settings across RPBG will be referred to as patients
- Personal Support Persons (PSPs), Next of Kin (NoK), family, carers, friends, a guardian or a nominated person will be referred to as support people.

Links within document

- [Appendix I: Multidisciplinary Comprehensive Care Plan Procedure](#)
- [Appendix II: Multidisciplinary Comprehensive Care Planning Workflow Variations](#)
- [Appendix III: Table of Comprehensive Care Document Types](#)
- [Appendix IV: Minimum Screening & Assessment Requirements](#)

Definitions

Comprehensive Care	Health care that is based on the patients needs and identified healthcare goals. These goals are aligned with the patient's expressed preferences and healthcare needs, consider the impact of the patient's health issues on their life and wellbeing, and are clinically appropriate. ¹
Risk Screening and Management	A process of identifying and managing/preventing clinical risks to a patient. ¹
What Matters Most (WMM) to the Patient	Involves identifying with the patient, their agreed goals, significant concerns and actions for their treatment and care. ¹
Shared Decision Making	A consultation process where clinicians and patients participate in making joint health decisions, having discussed the options, their benefits, and harms, having considered the patient's values, preferences and circumstances. ¹

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Policy directives

General

As part of health care delivery, all staff (within their scope of practice) are required to:

- Actively partner with and support patients (as appropriate) in shared decision making to ensure care is aligned with expressed goals of care, and considers cultural, spiritual, physical wellbeing needs and preferences
- Meet the patient/ support people's information/ education needs to understand and support the management of their own health care
- Collaborate with the Multidisciplinary team (MDT) to plan and deliver best practice, individualised interventions that incorporate the assessment and skills of the whole MDT, and the goals of the patient. The patients plan of care may include 'goals of care/ end of life' decisions
- Work collaboratively with volunteers/ peer support workers, advocates, Non-Government Organisations and other relevant external stakeholders, incorporating information and feedback into care planning and delivery
- Proactively address barriers to communication by adopting strategies that assist in mitigating health literacy, sensory, language, and/or cognitive barriers
- Promote multidisciplinary teamwork and collaboration at all stages of the patient journey by ensuring effective communication of critical information across the multidisciplinary team, clear documentation, and timely referrals.

These principles are applied to establish and maintain systems and processes which support clinicians to deliver CC to all patients.

Documentation

- All members of the multidisciplinary team are required to document their assessment findings, plans and interventions in the patient health record
- For inpatients, a minimum CC risk screening and assessment must be completed on a local [Multidisciplinary Comprehensive Care Plan \(MCCP\)](#) for each patient, to inform individualised interventions for minimising patient harm. This should be completed and documented within 10 hours of admission to the ward/clinic/unit (skin assessment must be completed and documented within 2 hours)
- All staff documenting in the *MCCP* should record their name, initials and designation in the staff record or signature log
- The *MCCP* should be accessible at the bedside (except in Mental Health, where the *MCCP* is kept in the patient's healthcare record so all staff members can see the information they need to provide patient care)
- To ensure comprehensive care continues throughout the patient journey, refer to the Clinical Handover Policy and relevant admission policies for information on documentation and patient handover and/or transfer requirements.

Communication to support teamwork, collaboration and partnership

Effective communication is essential for teamwork and the successful delivery of comprehensive care. Communication is the process of sharing information, ideas, and feedback among teams with patients and care givers. All members of the MDT are required to communicate and collaborate to deliver patient care. This includes communicating and documenting comprehensive care planning, changes in clinical condition, timely referrals, interventions, and discharge plans.

The following are some of the mechanisms used to effectively communicate:

- Clinical handover
- Journey board meetings
- Ward rounds
- Ad hoc MDT review (e.g. post incident huddles, change in clinical condition and critical incident debrief)
- Scheduled MDT meetings
- Safety huddles
- Patient education
- Patient and carer interview.

Staff are encouraged to use the following communication systems and tools to facilitate effective communication between MDT members. These include but are not limited to:

- Journey boards
- Digital Medical Record (DMR)
- Inpatient notes including progress notes, MCCP's, team proforma's etc.
- Electronic Booking System (EBS)
- Enterprise Bed Management (EBM)
- iSoft Clinical Manager iCM
- Goals of Patient Care (GoPC) eform
- MetaVision
- Psychiatric Services Online Information System (PSOLIS)/ WebPSOLIS
- eReferral
- My Health Record
- Intellispace Critical Care Anaesthesia (ICCA) HIVE
- MS teams.

Effective communication does not guarantee quality care – the information communicated within MDT's needs to be appropriately acted upon. Ineffective communication, particularly of critical information like patient risk or deterioration can lead to adverse events such as failure to rescue, inappropriate treatment, care that is not aligned with patient goals and/or preferences and poor coordination of care.

Departments should clearly articulate expectations of roles, responsibilities, and accountability for all team members. E.g. all ward patients should be reviewed by a senior clinical decision maker by 11am.

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Communication to support teamwork, collaboration and partnership cont'd

Where electronic communication tools (e.g. MS teams) are utilised, local processes are to be put in place to monitor and administer appropriate use and permissions. Consideration is to be given to ensure confidentiality and privacy risk. Where clinician to clinician collaboration includes a key decision, diagnosis or treatment decision this must be transferred to the patient's healthcare record as per the [Department of health Acceptable Use of Information and Communications Technology Policy](#) and [M365 Acceptable Use Guideline](#).

Refer to [Staff Use of Electronic Communication Devices in Clinical Areas Policy](#).

Consumer engagement & shared decision making

The MDT will:

- Identify and include:
 - The patient's needs, preferences, WMM to them
 - Goals of patient care, both non-palliative and palliative (where applicable)
 - The patient's nominated substitute decision-makers and/or support people and engage them in the process, facilitating meetings as required. Consider approaches that will successfully navigate potential conflicts between support people and family.
- Proactively address barriers to communication, including sensory impairments, language, culture and cognition
- Ensure patient capacity is considered in shared decision making. Refer to [Assessing Patient Capacity SOP](#) and local, relevant service SOPs/ policies.
- Partner with patient and support people where possible (with patient consent) to actively involve them in patient care and encourage and educate them on how to be actively involved in care decisions where appropriate. New critical information can come from patients and their support people.
- Utilise handover and other interactions to meet the patient's information needs, engaging patients and support people in the process (where appropriate).
- Apply the principles of [Choosing Wisely](#) when discussing treatment options
- Provide information to facilitate and advocate for informed consent.

Compliance monitoring

Auditing and monitoring against policy requirements for risk screening, assessment, care planning, shared decision making and delivery of interventions will be achieved through bimonthly organisation wide Combined Bedside Risk Assessment (CoBRA)

Comprehensive Audits.

Heads of Department/ Ward Nurse Unit Managers are responsible for supporting the implementation of this Policy including auditing compliance, monitoring results, providing staff with regular feedback on performance related to CC, and identifying action plans to support local improvements.

Any reviews or additions to CC documentation must include all related *MCCP's* (refer to [Appendix III: Table of Comprehensive Care Document Types](#)), this Policy and any area specific documents referencing CC.

Ownership of the CC documents (including CC SOP, *MCCP's* and associated CC Documents) falls with the Comprehensive Care Committee.

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Authors / acknowledgements

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Aboriginal Health Impact Statement and Declaration

The voice of the Aboriginal community is reflected in the Comprehensive Care Policy through consultation with RPBG Aboriginal Health Liaison Officer Coordinators and Liaison Officers and the Aboriginal Acute Care Coordination Team to ensure that cultural appropriateness and the health impacts on Aboriginal people have been considered and incorporated (ISD Record 1689).

Note: Within Western Australia, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

References

1. Australian Commission on Safety & Quality of Health Care. (2018). Implementing the Comprehensive Care Standard. Sydney. Australia.

Related policies, practice standards, clinical guidelines

RPBG Policy Hub

- [Clinical Handover Policy](#)
- [General Health Admission, Transfer and Discharge SOP](#)
- [Mental Health Inpatient Triage to Discharge](#)
- [Recognising and Responding to Acute Deterioration in Mental State Cognition & Behaviour Policy](#)
- [Staff Use of Electronic Communication Devices in Clinical Areas Policy.](#)
- [Recognising and Responding to Acute Deterioration Policy](#)

EMHS Policy Hub

- [Care Coordination in Mental Health Services Policy](#)

Department of Health Policy Hub

- [Triage to Discharge Mental Health Framework for SSCD](#)

Related forms

- Multidisciplinary Comprehensive Care Plan (MCCP)
- Refer to [Appendix III](#)

Related National Standards

ACSQHC NSQHS Standards 2nd Edition (2021)

Standard 1: Clinical Governance

Standard 5: Comprehensive Care

Appendix I: Multidisciplinary Comprehensive Care Plan Procedure

What Matters Most

Begin a new interaction by introducing yourself to the patient/family/carer using 'hello my name is'. All staff complete the WMM can identify and document WMM in the *WMM Record* or designated WMM sections in care plans. This will help capture important individualised information about a person in the context of their agreed goals, significant concerns and actions for their treatment and care. The *WMM record* is to be completed with patients and their support people, and accessible at the bedside.

WMM should be commenced on admission and reviewed throughout the patient journey.

Patients and/ or their nominated support people are encouraged to write responses with assistance from any MDT member as required.

Tools exist to guide conversation and documentation of WMM (e.g. [Patient Welcome Pack](#)).

Staff should record attempts made to support patients to complete the WMM record including when it could not be completed.

When reviewing a patient's care plan, staff should refer to the WMM. Reviewing and actioning WMM can strengthen your relationships with patient / support people

Risk Screening & Assessment

- The MDT is collectively responsible for screening and assessing risks and using findings to inform the development of an individualised care plan for the patient.
- Risk screening and assessment on *MCCP* must be completed within 10 hours of admission.
- Nursing will take the lead in completing the initial screening and assessment components of the *MCCP* used in the area.
- If the admitting staff member does not complete the *MCCP*, the next staff member to care for the patient must complete it.
- The appropriate *MCCP* for use and minimum risk screening requirements are outlined in [Appendix III: Table of Comprehensive Care Plan Types](#) (complete additional risk screening as appropriate to specialty, e.g., *MH RAMP*).

An assessment of active risks must be reflected in the active risks on admission section in the *MCCP*. The clinician completing the *MCCP* should place a tick in the column that best reflects their current clinical judgement of risk:

No - nil evidence

Yes - requires monitoring and intervention to minimise risk

Yes - definite risk requiring immediate intervention

Unable to confirm – specify

If a risk is identified or the 'unable to confirm' column is ticked, document relevant reasoning, assessment findings and/or observation in the space provided.

Patient Care Plan

The MDT are responsible for developing and maintaining an individualised patient care plan that addresses patient's goals/ preferences, active care needs and risk mitigation strategies based off the most recent risk assessment.

Adding or prescribing interventions:

- Interventions can be added or modified by any member of the MDT but must be communicated to other relevant MDT members
- Staff record their initials against custom interventions added and sign the Staff Record so that it is clear who prescribed it

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The WMM should be updated throughout the patient's admission. Relevant items are to be transcribed onto any visual aids in use (e.g. care board)

When working with a patient, including when doing a risk re-assessment, check the WMM and incorporate it into your care.

The MDT are required to incorporate WMM at handovers and in team discussions.

Actions and interventions associated with the WMM should be documented on the record and marked as complete once they are delivered/resolved.

When a continuation care plan is commenced, review the WMM record and incorporate it into care plan.

Patient Care Plan (continued)

Signing off on delivery of interventions:

- Initial the shift box once an intervention has been delivered.
- If the intervention is not delivered, leave the shift box blank.
- When an intervention is ceased, mark the shift box with 'X'
- When an intervention is not available mark the shift box with 'N'
- If a patient refuses an intervention mark the shift box with 'R'.

The referrals and clinical review record can be added to by any member of the MDT. It should be completed for ad-hoc or specialist reviews. It does not need to be completed for regular blanket reviews. The clinician responding to the referral is to sign that the referral has been completed.

Risk Re-Assessment

Risk re-assessments are documented in the *Risk Re-Assessment Record*. A Risk Re-Assessment must be completed:

- On ward transfer
- When a new risk factor is identified
- On condition change
- Post incident
- When patient / support person request
Any member of the MDT can initiate and/or contribute to the patient Risk Re-Assessment.
- A Risk Re-Assessment is guided by clinical findings like handover/documentation, patient presentation and MDT assessments
- For any new risk/s identified the MDT are to document findings and observations that validate the risk/s in space provided
- Additional interventions, plans and actions to address risks must be documented with the updated Risk Re-Assessment.
- Any Risk Re-Assessment must be communicated to the MDT, patient and support person

The MDT delivering patient care are responsible for working collaboratively to continually screen and assess the patient for active risks and update appropriate interventions in the Patient Care Plan to inform ongoing care, until discharge.

Multidisciplinary Continuation Care Plan

The *Multidisciplinary Continuation Care Plan* is started once a *MCCP* is completed (every 7 days).

The initial *MCCP* is to remain accessible at the bedside to provide baseline assessment to evaluate patient progress against. A Risk Re-Assessment must be completed on commencement of a *Continuation Care Plan*.

Appendix II: Multidisciplinary comprehensive care planning workflow variations

Refer to the [Appendix III: Table of Comprehensive Care Plans Types](#) and [Appendix IV: Minimum Screening & Assessment Requirements](#) for information on appropriate care plan use, risk screening/assessment inclusions/ exemptions, and accompanying documents.

Elective Surgery

Risk Screening & Assessment

A grey and white column key exists on the less than 23 hour *MCCP* and greater than 23 hour *MCCP* to help staff discern screening assessment requirements pre-procedure (grey) and on day of admission (white). On day of admission, staff must verify the pre-procedure assessment findings and complete remaining admission screens and assessments to ensure completion of all grey and white sections.

Coloured boxes appear in some care plans informing further action.

If amber box is ticked on the less than 23 hour *MCCP*, staff must notify theatre via phone of the amber finding.

Where a patient's procedure is cancelled and the patient has a booked surgery date within 3 months, staff can use the original *MCCP*. However, if a significant condition change has occurred between interactions, completion of a new *MCCP* is required.

What Matters Most

Staff document within the WMM section of the *MCCP*. If additional space is required, document in the integrated progress notes.

Day Procedure Area

Risk Screening & Assessment

In day procedure areas, patients/ support people are asked to self-complete a modified risk screening and assessment form. After verifying the patient's answers, staff will tick the corresponding checkbox.

Alternate assessments made by staff during verification should be documented in the notes. If staff complete the patient risk screening on the patient's behalf, staff should sign the declaration at the bottom of the patient section and indicate their relationship to the patient as 'nurse.' In this scenario, staff do not need to verify the answers.

Patient Care Plan

Interventions should be set by the admitting nurse. If no change to care requirements on shift change and/or subsequent presentations, staff can sign the signature log declaring no change in interventions required. If there is a change in risk or presentation, complete a risk re-assessment and mark relevant interventions on corresponding new care plan number that matches the risk re-assessment number.

What Matters Most

Staff document within the WMM section of the *MCCP*. If additional space is required, document in the integrated progress notes.

Appendix II: Multidisciplinary comprehensive care planning workflow variations cont'd

Mental Health

Risk Screening & Assessment

CC assessment in mental health has been modified from the *MCCP* used in other areas to minimise duplication from the Statewide Standardised Clinical Documentation (SSCD) risk requirements. Additional requirement for a review of previous PSOLIS Alerts.

Refer to the [Progressive Risk Assessment SOP](#) for information on the *Progressive Risk Assessment* and specific area inclusions/ exclusions.

Refer to [Mental Health Inpatient Triage to Discharge SOP](#) for additional MH assessments that will inform care planning (eg, SSCD) NOC/POC.

Additional Medical Consultant Review Outcomes Section

Refer to the [Minimum Requirement for Medical Review of Consumers Admitted to Inpatient Mental Health \(MH\) Wards or CMHS Policy](#) for information on this process.

What Matters Most

The WMM record should be commenced within 10 hours, updated throughout the patient's admission, and finalised within 24-48 hours using recovery focused language. The WMM informs the WebPSOLIS *Treatment, Support and Discharge Plan (TSDP)*. When staff are assisting the patient to prepare for discharge, they must complete the Things to Do / Check When I am Leaving Hospital section of the *WMM record*.

Emergency

Risk Screening & Assessment

All patients presenting via the emergency department will have the minimum required risk screening and assessment completed in the *Adult Triage Assessment form*. Specific patient groups on treatment pathways may have additional screening completed, e.g. patients attending EMW or using the NOF pathway.

Patient Care Plan

Care planning in emergency is captured in the medical and triage notes, speciality proforma's (e.g. AMU/ASU) and on the Emergency Department Information System (EDIS).

What Matters Most

WMM is not routinely documented for emergency patients unless they attend EMW, where staff document WMM in the Emergency Medicine Ward Proforma. However, WMM can be documented in the ED progress notes when indicated (e.g. consider during extended ED stay).

Appendix II: Multidisciplinary comprehensive care planning workflow variations
cont'd

Intensive Care Unit

ICU use an Electronic Medical Record (EMR) called MetaVision. The elements of CC are integrated digitally into the system.

Risk Screening & Assessment

Risk screening and assessment is completed on digital templates in the nursing progress notes section of MetaVision

Patient Care Plan

The MDT handover form within MetaVision pulls together key MDT assessment findings, interventions and plans for the patient in a central visible screen. Longer stay patients may also have 'Complex Case Meeting' notes completed.

What Matters Most

In MetaVision, staff can document and update actions and interventions associated with WMM to the patient using the WMM button found on progress notes, and/or the MDT handover form

Midwifery Birth Centre (MBC)

Risk Screening & Assessment

MBC risk screening is spread across the continuum of pregnancy, intrapartum and postnatal care. Antenatal screening tools are located within the West Australian Handheld Pregnancy Record. The admitting midwife will confirm that there are no changes to the risk level on admission for labour and birth using risk assessment and care planning tools on the partogram.

What Matters Most

Midwives will capture WMM details within the Western Australian (WA) Handheld Pregnancy Record during the antenatal period and document specific details on the Birth Plan/Tool Kit for use during the admission for labour and birth care. Any key pieces of information from WA Handheld Pregnancy Record will be transcribed onto the partogram during the intrapartum period.

Next Step Inpatient Withdrawal Unit (IPWU)

Risk Screening & Assessment

The risk screening and assessment in Next Step's IPWU is modified for drug and alcohol as their service specialises in managing drug and alcohol withdrawal and does increased assessment based off patient need. The service has inclusion/exclusion criteria. As a result, the venous thromboembolism and infection screening is modified to reflect their patient cohort and there is also no dysphagia/aspiration risk screening completed.

What Matters Most

The What Matters Most record should be completed during the patient's admission in collaboration with the patients and updated throughout their stay as required.

Appendix III: Table of comprehensive care document types

Wards	Care Plan Type	Minimum Screening & Assessment Reference in Appendix IV
BHS: All medical inpatient units RPH: All medical and long stay surgical units.	Multidisciplinary Comprehensive Care Plan (MCCP)	Risk Assessment (RA) 1
BHS: Elective units RPH: 4A/B, 3H	Elective Surgery >23 Hour Multidisciplinary Comprehensive Care Plan	RA1
BHS: All medical inpatient units, Elective Units RPH: All medical and long stay surgical units, 4A/B, 3H	Multidisciplinary Comprehensive Care Plan Continuation	RA1
All RPBG Mental Health units	Mental Health Multidisciplinary Comprehensive Care Plan	RA2
All RPBG Medical, Surgical, Next Step inpatient units	WMM Record	
All RPBG Mental Health units	Mental Health WMM Record	
BHS: All Mental Health units RPH: MHU, 2K Transitional Care Unit (TCU)	Mental Health Multidisciplinary Comprehensive Care Plan Continuation	RA2
TCU	My Better Life Plan & Client Wellness and Safety Plan	
BHS: Pre-Admissions, elective units RPH: 4A/B, DSU	Elective Surgery <23 Hour Multidisciplinary Comprehensive Care Plan	RA3
RPH: MDU, GIC, Dialysis Day procedural area's	Multi-Visit Day Procedures Multidisciplinary Comprehensive Care Plan	RA4
BHS: Endoscopy Ward RPH: Gastroenterology Day Ward	Endoscopy Day Procedures Multidisciplinary Comprehensive Care Plan	RA4
RPH: Cardiac Catheterisation Lab, 4F	Cardiac Day Procedures Multidisciplinary Comprehensive Care Plan	RA4
BHS: Electroconvulsive Therapy Unit (ECT)	ECT Day Procedures Multidisciplinary Comprehensive Care Plan	RA5
RPH: ED	Adult Triage Nursing Assessment	RA6
BHS: Nil RPH: EMW	Emergency Medicine Ward Admission Proforma	RA6
RPH: Ambulatory Unit (AU)	- Ambulatory Unit Multidisciplinary Admission Form - Ambulatory Unit Multidisciplinary Comprehensive Care Assessment Form - Ambulatory Unit Nursing Assessment Form	RA7
Next Step Inpatient Withdrawal Unit	Next Step Multidisciplinary Comprehensive Care Plan	RA8
BHS: Maternity	Maternity Multidisciplinary Comprehensive Care Plan	RA9

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Appendix IV: Minimum Screening & Assessment Requirements

Approach Label	Minimum Requirements
Risk Assessment (RA) 1	Standardised: absconding, alcohol/ drug/ nicotine withdrawal, communication, delirium/ cognitive impairment, dysphagia/ aspiration, falls, harm from others, harm to others, harm to self, infection, malnutrition, pressure injury, VTE.
RA2	Standardised: absconding, alcohol/ drug/ nicotine withdrawal, communication, delirium/ cognitive impairment, dysphagia/ aspiration, falls, infection, malnutrition, pressure injury, VTE. Modified: harm from others, harm to others, harm to self, absconding, alcohol/ drug/ nicotine risks covered by mandatory MH forms and screening processes.
RA3	Standardised: absconding, alcohol/ drug/ nicotine withdrawal, communication, delirium/ cognitive impairment, falls, harm from others, harm to others, harm to self, infection, malnutrition, VTE. Modified: pressure injury assessment. Excludes dysphagia/ aspiration assessment.
RA4 (Patient led with staff cross check)	Standardised: Absconding, alcohol/ drug/ nicotine withdrawal, communication, delirium/ cognitive impairment, falls, harm from others, harm to others, harm to self, infection, malnutrition, pressure injury, VTE. Modified: malnutrition and infection prevention screen. Excludes dysphagia/ aspiration assessment.
RA5 (ECT)	Standardised: absconding, alcohol/ drug/ nicotine withdrawal, communication, delirium/ cognitive impairment, falls, harm from others, harm to others, harm to self, infection, malnutrition, pressure injury. Modified: infection prevention screen. Modified harm from others, harm to others, harm to self, absconding, alcohol/ drug/ nicotine risks covered by mandatory MH forms and screening processes. Excludes dysphagia/ aspiration assessment
RA6 (ED, EMW)	Modified: Absconding, alcohol/ drug/ nicotine withdrawal, communication, delirium/ cognitive impairment, falls, harm from others, harm to others, harm to self, infection, pressure injury, VTE. Excludes dysphagia/ aspiration.
RA7 (Ambulatory Unit)	Modified: Communication, delirium/ cognitive impairment, falls, harm from others, harm to other, harm to self, infection, pressure injury. Excludes dysphagia/ aspiration.
RA8	Standardised: Absconding, communication, delirium/ cognitive impairment, falls, harm from others, harm to others, harm to self, malnutrition and pressure injury. Modified: Alcohol/ drug/ nicotine withdrawal, infection, VTE. Excludes dysphagia/ aspiration.
RA9	Modified: absconding, alcohol/ drug/ nicotine withdrawal, communication, delirium/ cognitive impairment, falls, harm from others, harm to others, harm to self, infection, malnutrition, pressure injury, VTE.

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